

PRODUCT NAME		Torzolp	COUNTRY : Malaysia	Supersedes A/W No.:		LOCATION : Indrad	
ITEM / PACK		Insert	NO. OF COLORS: 1	REMARK :			
DESIGN STYLE		Front-Back	PANTONE SHADE NOS.:	SUBSTRATE :			
CODE		xxxxxxx-5253	Black	Activities	Department	Name	Date
DIMENSIONS (MM)		150 x 240		Prepared By	Pkg.Dev		
ART WORK SIZE		S/S		Reviewed By	Pkg.Dev		
DATE		18-03-2020		Approved By	Quality		

xxxxxxx-5253
For the use of a Registered Medical Practitioner or a Hospital or a Laboratory

TORZOLP TABLETS 10 MG

(Zolpidem Tartrate Tablets U.S.P. 10 mg)

COMPOSITION

Each film coated tablet contains
Zolpidem Tartrate U.S.P. 10 mg
Excipients q.s.

Colour: Ferric oxide red, Ferric oxide yellow and Titanium dioxide

DESCRIPTION

Yellow coloured, capsule shaped tablets with 'Torrent logo' debossing on one side and '10 MG' debossing on another side.

CLINICAL PHARMACOLOGY

Pharmacodynamics:

Subunit modulation of the GABA_A receptor chloride channel macromolecular complex is hypothesized to be responsible for sedative, anticonvulsant, anxiolytic, and myorelaxant drug properties. The major modulatory site of the GABA_A receptor complex is located on its alpha (α) subunit and is referred to as the benzodiazepine (BZ) or omega (ω) receptor. At least three subtypes of the (ω) receptor have been identified.

Zolpidem, the active moiety of zolpidem tartrate, is a hypnotic agent with a chemical structure unrelated to benzodiazepines, barbiturates, pyrrolopyrazines, pyrazolopyrimidines or other drugs with known hypnotic properties, it interacts with a GABA-BZ receptor complex and shares some of the pharmacological properties of the benzodiazepines. In contrast to the benzodiazepines, which non-selectively bind to and activate all BZ receptor subtypes, zolpidem *in vitro* binds the (BZ₁) receptor preferentially with a high affinity ratio of the alpha₁/alpha₅ subunits. The (BZ₁) receptor is found primarily on the Lamina IV of the sensorimotor cortical regions, substantia nigra (pars reticulata), cerebellum molecular layer, olfactory bulb, ventral thalamic complex, pons, inferior colliculus, and globus pallidus. This selective binding of zolpidem on the (BZ₁) receptor is not absolute, but it may explain the relative absence of myorelaxant and anticonvulsant effects in animal studies as well as the preservation of deep sleep (stages 3 and 4) in human studies of zolpidem at hypnotic doses.

Pharmacokinetics:

Absorption

- Zolpidem is rapidly absorbed from the gastrointestinal tract after oral doses, peak plasma concentrations occurring within 3 hours.
- Bioavailability, Oral, tablets: 70%
- Effect of food: decreased systemic exposure (decreased C_{max} and AUC; increased T_{max}).

Distribution

- Zolpidem is about 92% bound to plasma proteins.
- The distribution volume in adults is 0.54L/kg.
- Zolpidem is distributed into breast milk.

Metabolism

- Liver, extensive.
- Zolpidem undergoes first-pass metabolism. It is metabolised mainly by the cytochrome P450 isoenzyme CYP3A4.

Elimination

- Zolpidem has an elimination half-life of about 2.5 hours. The inactive metabolites of zolpidem are excreted in the urine and faeces.
- Zolpidem is not dialysable.

Populations at risk

- In patients with renal insufficiency, whether dialysed or not, a moderate decrease in clearance is observed. Other kinetic parameters remain unchanged.
- In patients with hepatic insufficiency, the bioavailability of zolpidem is increased.
Clearance is reduced and elimination half-life prolonged (about 10 hours).

INDICATIONS AND USAGE:

Zolpidem tartrate tablets are indicated for treatment of severe sleep disorders in the following cases: Occasional and transient insomnia.

CONTRAINDICATIONS:

Zolpidem tartrate tablets are contraindicated in patients with known hypersensitivity to zolpidem tartrate or to any of the inactive ingredients in the formulation. Observed reactions include anaphylaxis and angioedema.

Severe respiratory insufficiency, sleep apnoea syndrome, severe, acute or chronic hepatic insufficiency (risk of encephalopathy), myasthenia.

Due to the lactose content in TORZOLP, it is contraindicated in the event of congenital galactosaemia, glucose or galactose malabsorption syndrome or lactase deficiency.

WARNINGS

- ANAPHYLAXIS (SEVERE ALLERGIC REACTION) AND ANGIOEDEMA (SEVERE FACIAL SWELLING) WHICH CAN OCCUR AS EARLY AS THE FIRST TIME THE PRODUCT IS TAKEN**
- COMPLEX SLEEP RELATED BEHAVIORS WHICH MAY INCLUDE SLEEP DRIVING, MAKING PHONE CALLS, PREPARING AND EATING FOOD WHILE ASLEEP**

Anaphylaxis (severe allergic reaction) and angioedema (severe facial swelling) which can occur as early as the 1st time TORZOLP is taken.

Tolerance: After repeated administration over several weeks, the sedating or hypnotic effect of benzodiazepines and related substances may gradually decrease.

Dependence: Any administration of benzodiazepines and related substances, particularly prolonged use, may induce the development of physical or psychic drug dependence.

Several factors appear to favour onset of drug dependence: Treatment duration, dosage, history of dependence to other drugs (medicinal products or not) and alcohol. Drug dependence may appear at therapeutic dose levels and/or in patients not presenting with individual risk factors.

Exceptional cases of drug dependence have been reported for therapeutic doses of zolpidem.

This state of dependence can produce withdrawal syndrome when treatment is discontinued.

Some symptoms are frequent and appear commonplace: Insomnia, headaches, marked anxiety, myalgia, muscle tension and irritability. Other symptoms are rarer: Agitation or even confusion, paresthesia of fingers and toes, hyper-reactivity to light, sound and physical contact, depersonalization, derealization, hallucinations and convulsions.

Withdrawal symptoms may appear during the days following treatment discontinuation. With short-acting benzodiazepines, certain withdrawal symptoms may occur between 2 consecutive intakes, especially at high doses.

Whatever the anxiolytic or hypnotic indication, the combination of several benzodiazepines is liable to increase the risk of drug dependence.

Cases of drug abuse have been reported.

Rebound Insomnia: This transient syndrome is a worsening of the initial insomnia for which the benzodiazepines and related substances were prescribed.

Amnesia: Anterograde amnesia and impaired psychomotor function may occur within hours of intake. To reduce these risks, the product should be taken just before going to bed, or even in bed and in optimal conditions for uninterrupted sleep of 7-8 hrs.

Behaviour Disorders: Benzodiazepines and related substances may induce syndrome that combine impaired consciousness and behaviour and memory disorders at varying degrees.

The following may be observed: Worsening of insomnia, nightmares, agitation, nervousness; delirium, hallucinations, oniric delirium, psychotic symptoms; disinhibition with impulsiveness; euphoria, irritability; anterograde amnesia; suggestibility.

This syndrome may be accompanied by disorders that could be potentially dangerous for the patient and others eg, unusual patient behaviour patterns; aggressiveness to self and others, especially if the patient is prevented from doing something he/she wants to do; automatic behaviour with post-event amnesia.

If these symptoms occur, treatment should be discontinued. These symptoms are more often observed in elderly subjects.

Somnambulism and Associated Behaviours: Sleep-walking associated with other behavior, eg, night-time driving, cooking and eating, using the telephone or engaging in sexual intercourse, with subsequent amnesia, has been reported in patients who had taken zolpidem and were not fully awake.

The use of alcohol and other central nervous system depressants with zolpidem seems to increase the risk of such behaviour, as does administration of zolpidem at doses higher than the maximum recommended dose.

Discontinuation of zolpidem treatment is strongly recommended in patients who have experienced such behaviour.

Elderly Patients: Great care must be taken when prescribing benzodiazepines and related substances to elderly patients, because of the risk of sedation and/or muscle relaxant effects which may lead to falls with serious consequences in this population

PRECAUTIONS

Great care is required in the event of history of alcoholism or drug addiction (medicinal products or not). In all cases, insomnia must systematically be assessed and its causes treated before a hypnotic is prescribed. Insomnia may be related to an underlying physical or psychiatric disorder. Persistence or worsening of insomnia after a short treatment period requires further clinical assessment.

Patients with Major Depression: Since insomnia may be a symptom of depression, the depression must be treated. If insomnia persists, the patient's condition must be reassessed.

In subjects with a major depressive episode: Benzodiazepines and related substances should not be used alone since they enable depression to evolve with persistence or worsening of suicidal tendencies.

As there is a risk of suicide in these patients, the lowest amount of zolpidem must be prescribed and supplied to them in order to limit the possibility of intentional overdose.

Gradual Discontinuation of Treatment: The patient must be clearly informed of treatment discontinuation modalities.

As well as knowing about gradual discontinuation of treatment, patients must be warned of the possibility of rebound insomnia, so that potential insomnia resulting from symptoms related to such discontinuation, even if gradual, may be minimized. The patient must

be informed that this period may be difficult.

Children: In the absence of reported clinical studies, Zolpidem should not be given to children <18 years.

Elderly Patients and Patients with Hepatic Insufficiency: Dosage should be reduced (ie, by half) because of the risk of accumulation. Patients with Respiratory Insufficiency: In patients with respiratory insufficiency, the depressant effect of benzodiazepines and related substances must be taken into account (especially since anxiety and agitation may be signs of respiratory function decompensation requiring hospitalization in an intensive care unit).

Effects on the Ability to Drive or Operate Machinery: Drivers and machine operators should be informed of the risk of drowsiness associated with the use of Zolpidem. The risk of impaired vigilance is exacerbated if patients do not get sufficient sleep.

PREGNANCY AND LACTATION

Pregnancy Category C

Use in pregnancy & lactation: As a precautionary measure, do not use zolpidem at any stage during pregnancy.

Breastfeeding is not recommended during treatment

DRUG INTERACTIONS:

CNS-active drugs

Since the systematic evaluations of zolpidem in combination with other CNS-active drugs have been limited, careful consideration should be given to the pharmacology of any CNS-active drug to be used with zolpidem. Any drug with CNS-depressant effects could potentially enhance the CNS-depressant effects of zolpidem.

Drugs that affect drug metabolism via cytochrome P450

Some compounds known to inhibit CYP3A may increase exposure to zolpidem. The effect of inhibitors of other P450 enzymes has not been carefully evaluated.

Other drugs with no interaction with zolpidem

A study involving cimetidine/zolpidem and ranitidine/zolpidem combinations revealed no effect of either drug on the pharmacokinetics or pharmacodynamics of zolpidem.

Zolpidem had no effect on digoxin pharmacokinetics and did not affect prothrombin time when given with warfarin in normal subjects.

SIDE EFFECTS

The following serious reactions are discussed in greater detail in other sections of the labeling:

- Serious anaphylactic and anaphylactoid reactions
- Abnormal thinking, behavior changes, and complex behaviors
- Withdrawal effects
- CNS-depressant effects

Autonomic nervous system: Infrequent: increased sweating, pallor, postural hypotension, syncope. Rare: abnormal accommodation, altered saliva, flushing, glaucoma, hypotension, impotence, increased saliva, tenesmus.

Body as a whole: Frequent: asthenia. Infrequent: edema, falling, fatigue, fever, malaise, trauma. Rare: allergic reaction, allergy aggravated, anaphylactic shock, face edema, hot flashes, increased ESR, pain, restless legs, rigors, tolerance increased, weight decrease.

Cardiovascular system: Infrequent: cerebrovascular disorder, hypertension, tachycardia. Rare: angina pectoris, arrhythmia, arteritis, circulatory failure, extrasystoles, hypertension aggravated, myocardial infarction, phlebitis, pulmonary embolism, pulmonary edema, varicose veins, ventricular tachycardia.

Central and peripheral nervous system: Frequent: ataxia, confusion, euphoria, headache, insomnia, vertigo. Infrequent: agitation, anxiety, decreased cognition, detached, difficulty concentrating, dysarthria, emotional lability, hallucination, hypoesthesia, illusion, leg cramps, migraine, nervousness, paresthesia, sleeping (after daytime dosing), speech disorder, stupor, tremor. Rare: abnormal gait, abnormal thinking, aggressive reaction, apathy, appetite increased, decreased libido, delusion, dementia, depersonalization, dysphasia, feeling strange, hypokinesia, hypotonia, hysteria, intoxicated feeling, manic reaction, neuralgia, neuritis, neuropathy, neurosis, panic attacks, paresis, personality disorder, somnambulism, suicide attempts, tetany, yawning.

Gastrointestinal system: Frequent: dyspepsia, hiccup, nausea. Infrequent: anorexia, constipation, dysphagia, flatulence, gastroenteritis, vomiting. Rare: enteritis, eructation, esophagospasm, gastritis, hemorrhoids, intestinal obstruction, rectal hemorrhage, tooth caries.

Hematologic and lymphatic system: Rare: anemia, hyperhemoglobinemia, leukopenia, lymphadenopathy, macrocytic anemia, purpura, thrombosis.

Immunologic system: Infrequent: infection. Rare: abscess, herpes simplex, herpes zoster, otitis externa, otitis media.

Liver and biliary system: Infrequent: abnormal hepatic function, increased SGPT. Rare: bilirubinemia, increased SGOT.

Metabolic and nutritional: Infrequent: hyperglycemia, thirst. Rare: gout, hypercholesteremia, hyperlipidemia, increased alkaline phosphatase, increased BUN, periorbital edema.

Musculoskeletal system: Frequent: arthralgia, myalgia. Infrequent: arthritis. Rare: arthrosis, muscle weakness, sciatica, tendinitis.

Reproductive system: Infrequent: menstrual disorder, vaginitis. Rare: breast fibroadenosis, breast neoplasm, breast pain.

Respiratory system: Frequent: upper respiratory infection. Infrequent: bronchitis, coughing, dyspnea, rhinitis. Rare: bronchospasm, epistaxis, hypoxia, laryngitis, pneumonia.

Skin and appendages: Infrequent: pruritus. Rare: acne, bullous eruption, dermatitis, furunculosis, injection-site inflammation, photosensitivity reaction, urticaria.

Special senses: Frequent: diplopia, vision abnormal. Infrequent: eye irritation, eye pain, scleritis, taste perversion, tinnitus. Rare: conjunctivitis, corneal ulceration, lacrimation abnormal, parosmia, photopsia.

Urogenital system: Frequent: urinary tract infection. Infrequent: cystitis, urinary incontinence. Rare: acute renal failure, dysuria, micturition frequency, nocturia, polyuria, pyelonephritis, renal pain, urinary retention.

OVERDOSAGE:

Signs and symptoms

In postmarketing experience of overdose with zolpidem tartrate alone, or in combination with CNS-depressant agents, impairment of consciousness ranging from somnolence to coma, cardiovascular and/or respiratory compromise, and fatal outcomes have been reported.

Recommended treatment

General symptomatic and supportive measures should be used along with immediate gastric lavage where appropriate. Intravenous fluids should be administered as needed. Zolpidem's sedative hypnotic effect was shown to be reduced by flumazenil and therefore may be useful; however, flumazenil administration may contribute to the appearance of neurological symptoms (convulsions). As in all cases of drug overdose, respiration, pulse, blood pressure, and other appropriate signs should be monitored and general supportive measures employed. Hypotension and CNS depression should be monitored and treated by appropriate medical intervention. Sedating drugs should be withheld following zolpidem overdose, even if excitation occurs. The value of dialysis in the treatment of overdose has not been determined, although hemodialysis studies in patients with renal failure receiving therapeutic doses have demonstrated that zolpidem is not dialyzable.

As with the management of all overdose, the possibility of multiple drug ingestion should be considered. The physician may wish to consider contacting a poison control center for up-to-date information on the management of hypnotic drug product overdose.

DOSAGE AND ADMINISTRATION:

Recommended dose

The treatment should always be implemented at the lowest effective dose and maximum dosage never exceeded.

The usual dose for adults is one 10-mg tablet daily.

The medicinal product should always be taken just before going to bed.

In elderly subjects or subjects presenting with hepatic insufficiency: dosage should be halved, i.e. 5mg.

Dosage must never exceed 10 mg per day.

Safety and effectiveness of zolpidem in paediatric patients under the age of 18 years have not been established. Therefore, zolpidem should not be prescribed in this population.

Zolpidem can be prescribed either continuously or on demand, depending on the patient's symptoms.

Treatment duration

The treatment period should be as short as possible, from a few days to four weeks, including the tapering period.

The patient should be advised to take the treatment as follows:

- 2 to 5 days for occasional insomnia (e.g. during a trip)
- 2 to 3 weeks for transient insomnia (e.g. during a troubled period).

Very short treatment periods do not require any gradual treatment discontinuation.

For certain patients, it may be necessary to continue treatment for longer than four weeks, in which case careful and repeated reassessment of the patient's condition is necessary.

Mode of Administration: Oral

STORAGE:

Store below 30°C.

PRESENTATIONS:

Zolpidem Tartrate Tablets are packed in Alu-Alu blister containing 10 tablets in each blister. Such blister strips are packed into box of 30's and 100's.

DATE OF REVISION

13th March, 2020



Manufactured by :

TORRENT PHARMACEUTICALS LTD.

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